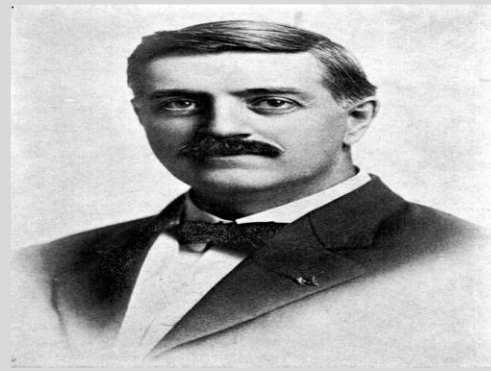


Session Objectives

- Understand that the neurophysiologic mechanism of Chapman's points involves viscerosomatic reflexes
- Relate Chapman's reflexes to the neuroendocrine immune response.
- Describe the common manifestations of endocrine diseases as they relate to Chapman's reflexes.
- Know that Chapman's reflexes involve the sympathetic arm of the autonomic nervous system.
- Use memory tools for Chapman's points such as high density areas like the second intercostal space and the lateral thigh.
- Incorporate Chapman's reflexes into the five models of osteopathic medicine.



Frank Chapman, D.O.



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- At the core of classic osteopathy as taught by his teacher, A.T. Still, was the concept of diagnosing autonomic tone by the “feel of the tissues” and recognizing the unlimited potential of the lymphatics.
- Dr. Frank Chapman practiced and charted his “lymphatic reflexes” for over thirty years.
- Upon Dr. Frank Chapman’s death, his wife, Dr. Ada Hinckley Chapman, and his brother-in-law, Dr. Charles Owens along with Dr. W.F. Link assembled his writings into “An Endocrine Interpretation of Chapman’s Reflexes”. This set the stage for the concept of unity of hormones, immunity and the autonomic nervous system.



High Density Areas of Chapman's Reflexes

2nd Intercostal Space (ICS):

- Bronchus (B)
- Esophagus (E)
- Thyroid (T)
- Heart (H)
- B.E.T.H.
- Clinical correlations include a patient with bronchitis who is status post MI with a history of GERD and Hashimoto's thyroiditis



High Density Areas of Chapman's Reflexes, II

Lateral Femur (Lateral thigh)

- Broad Ligament-Outer Femur (B)
- Prostate-Outer Femur (P)
- Colon-Anterior Outer Femur (C)
- “Sciatic Neuritis” (aka Lumbosacral radiculopathy)-Posterior Outer Femur (S)

“Back Pain Causes Spasm”



From: Muscle Energy

An Osteopathic Approach to Diagnosis and Treatment, 3e, 2004



Legend: The Muscle Energy model of innominate correction, especially position, used even in a passive fashion, would suffice to correct a pelvic dysfunction. The heritage of Dr. Fred Mitchell, Sr and Jr. to the Chapman's model goes beyond a power point presentation



Case Study

Nephrotic Syndrome

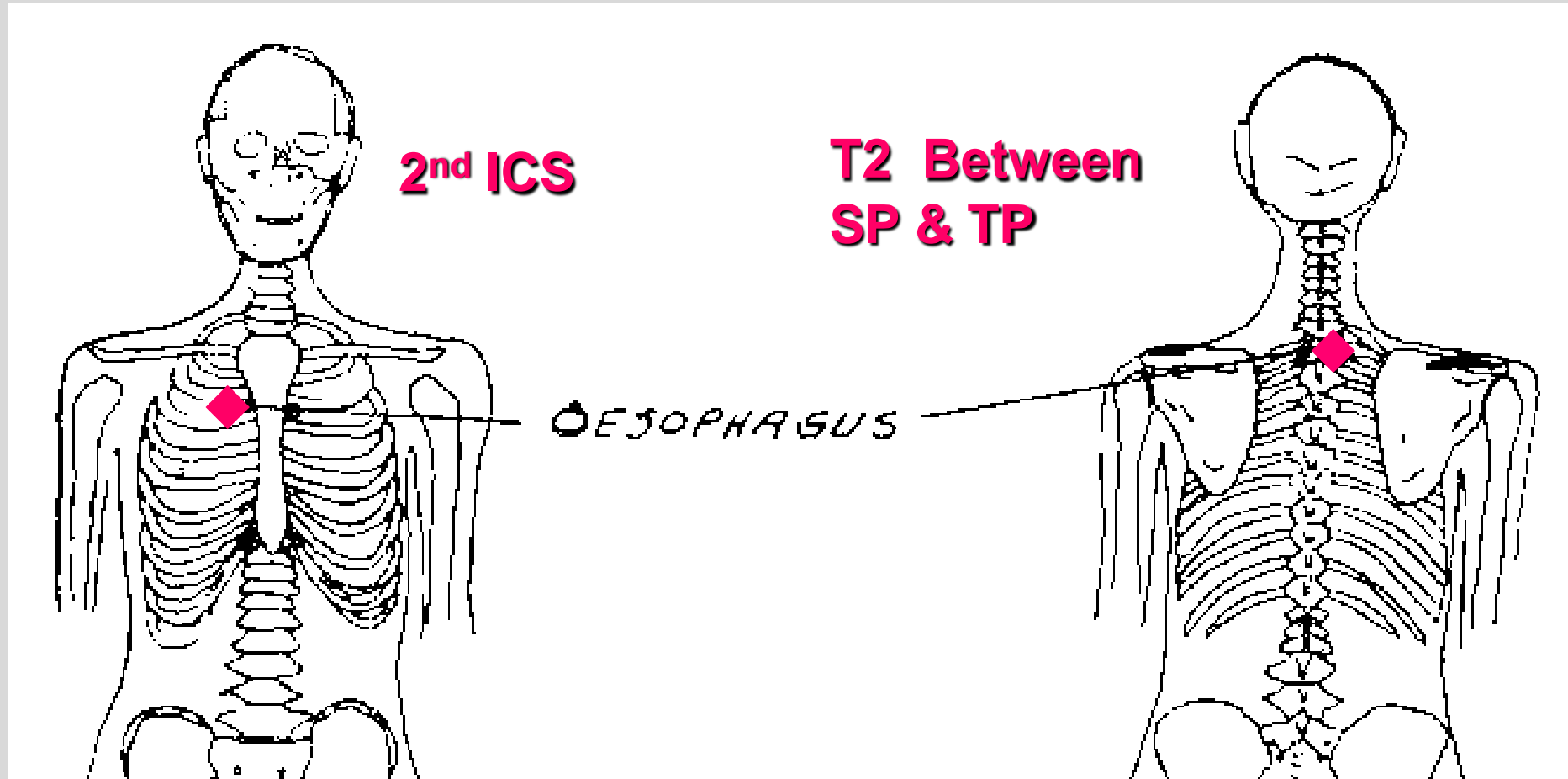
- 19 yr. Old AA female with nephropathy who presented with oliguria and peripheral edema.
- Unresponsive to diuretics, albumin and dietary restrictions.
- Daily urine output about 250 cc's
- OMT given at day 5 of hospitalization.
- OMT directed to lymph, diaphragms and Chapman's point at the periumbilical and thoracolumbar regions
- Immediate 400 cc's diuresis within 45 minutes.
- Next three days patient voided about 1500-2700 cc's of urine.
- Avoided cyclophosphamide.
- Patient was discharged and treated as an outpatient on day 8.



Esophagus (GERD)

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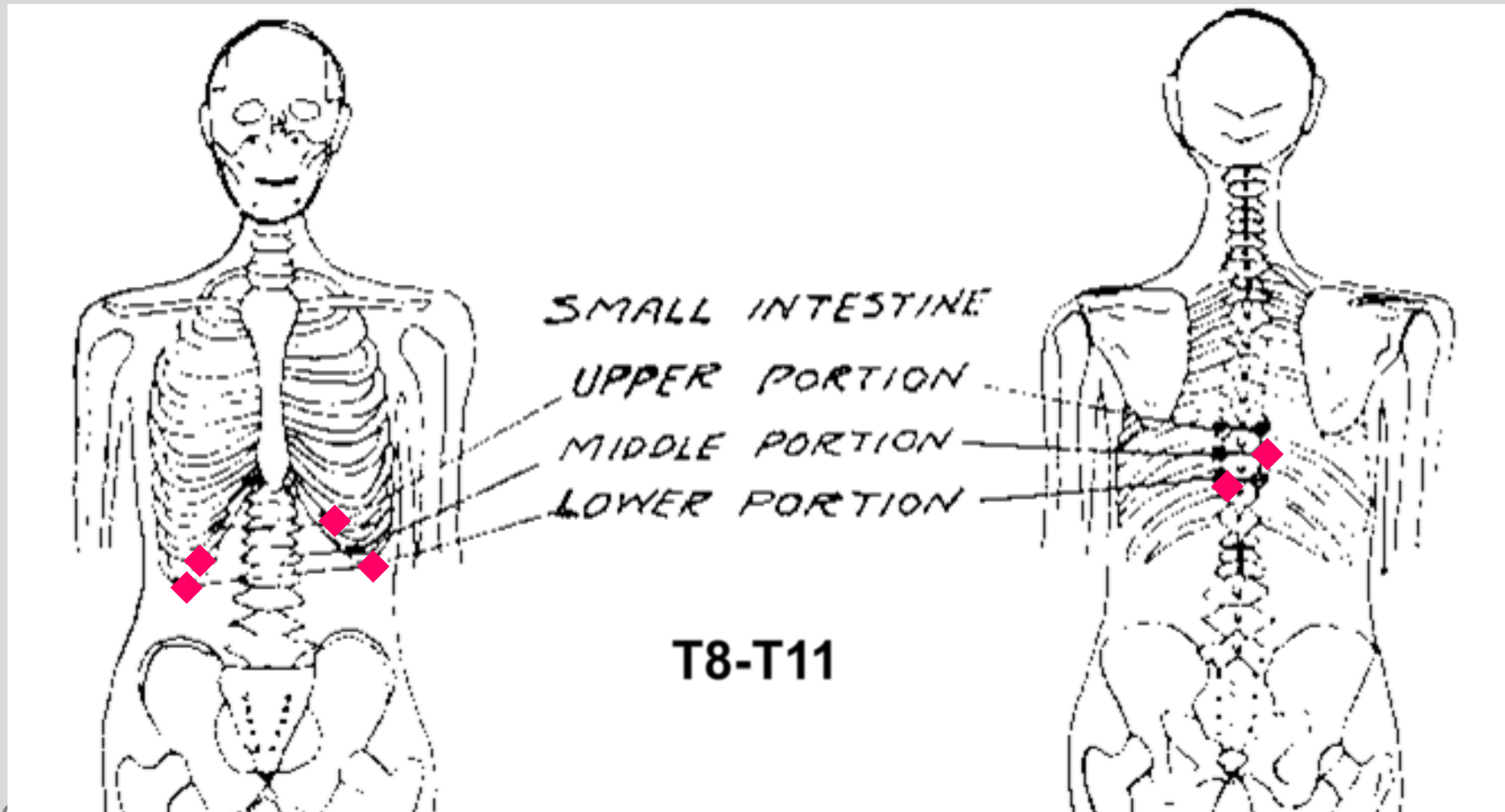
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Small Intestines

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Anterior
8,9,10 ICS

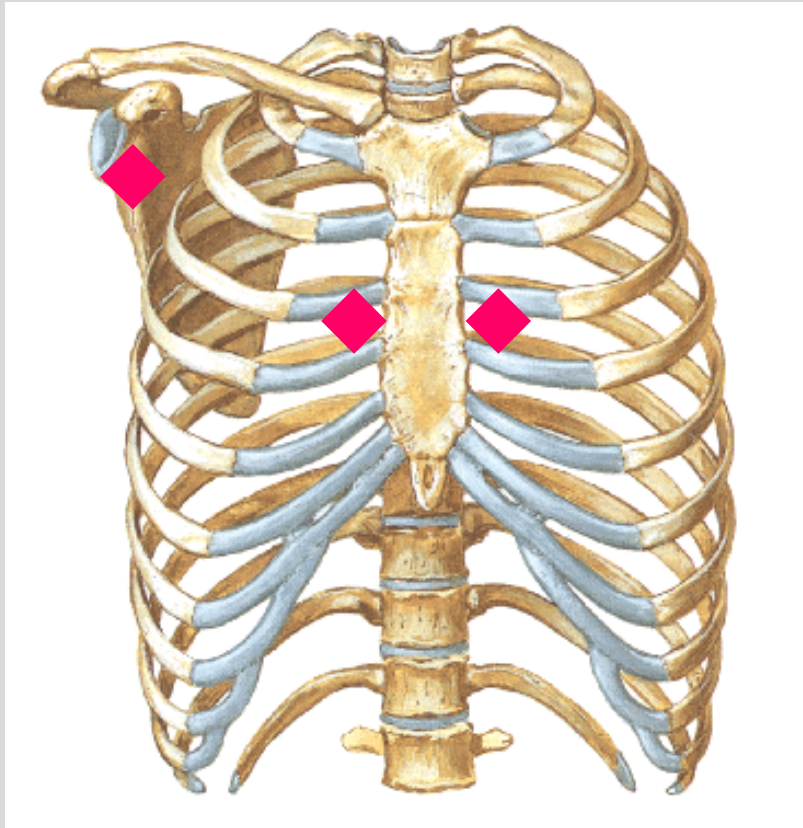
Posterior
Intertransverse
space 8-9, 9-
10, 10-11



Neuritis of Upper Extremity

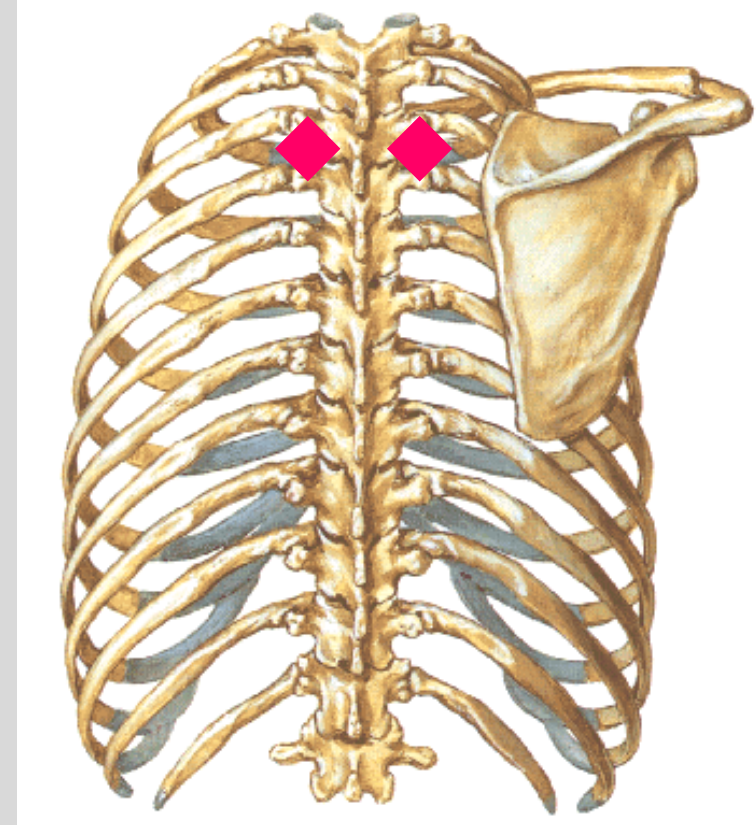
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**3rd Rib &
Parker's Reflex**

Department of Osteopathic Manipulative Medicine



Between TP's of T3 & T4



PTAS: Pelvic-Thyroid-Adrenal Syndrome: The Neuro-Endocrine Immune Response: NEI

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- Chapman's PTAS and its interaction with the neuroendocrine immune response is first treat the pelvis because that is where the gonads and adrenals are housed—they emit hormones such as estrogen, progesterone, testosterone, cortisol and norepinephrine for example. These organs are innervated by the sympathetics. There is a feedback cycle with thyroxine, a thyroid hormone which has widespread effects throughout the entire body, including on the immune system.
- An example of the PTAS/NEI response would be thyrotropin and thyroxine stimulates natural killer cells and produces T-cells respectively. Stress, or the musculoskeletal ready state, is mediated by the sympathetic nervous system and promotes release of cortisol from the adrenal glands, which also modulates immunocytes.

Capobianco, John D., The Neuroendocrine-immune complex as illustrated in the work of Dr. Frank Chapman. J Am Acad Osteopath. 2004; 14(1)



Chapman's Quick Reference

HEENT

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Organ/Disease Examples	Anterior and Posterior Locations
Brain-Cerebrum/CVA/Paralysis New: Insomnia-Surface of 4 th Rib underlying scapula	A: Lateral aspect of spinous processes of C3-5 P: Between C1-2 t-processes
Brain-Cerebellum/Confusion/Childhood Learning	A: Corocoid process P: C1 t-process
Ears/Otitis media/Motion Sickness/Mastoiditis/Vertigo	A: Medial-superior clavicle P: C1 t-process
Eyes/Conjunctivitis/Retinitis	A: Surgical neck of humerus P: Occiput-suboccipital nerve
Nose/Rhinitis/Hay Fever	A: 1 st rib & cartilage P: Lateral C1 from line of jaw angle
Neurasthenia/Fatigue/Decreased Mental Alertness/Weakness Especially after Exertion. Associated with thyroid, heart, GI and Reproductive symptoms	A: Pectoralis major from humerus, clavicle, sternum, cartilage, especially down to fourth rib in midaxillary line P: 4 th Rib under scapula (associated with insomnia)
Pharynx-Eustachian tube-Uvula/Pharyngitis	A: 1 st rib $\frac{3}{4}$ inch from sternum P: Trans. Process of C2

Chapman's Quick Reference HEENT (Continued)

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Organ/Disease Examples	Anterior and Posterior Points
Tonsils/Tonsillitis-Strep throat	A: 1 st Intercostal space (ICS) P: Trans. Process C1
Tongue/Choking/Soreness	A: 2 nd rib cartilage $\frac{3}{4}$ inch from sternum P: Between spinous and transverse process of C2
Larynx/Laryngitis/Aphonia	A: 2 nd rib-superior aspect- $\frac{3}{4}$ inch from sternum P: Between spinous & transverse process of C2
Paranasal Sinuses/Sinusitis	A: 1 st ICS & 2 nd rib $\frac{3}{4}$ inch from sternum P: Between spinous & transverse process of C2
New Sinus Points	Fourth rib and lateral edge of scapula
New HEENT Congestion	2nd rib in axilla
New Acne/Psoriasis Points	3 rd and 4 th ribs posteriorly
New Epistaxis Point	Suboccipital nerve on occiput

Chapman's Quick Reference Cardiopulmonary

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Organ/Disease Examples	Anterior and Posterior Points
Heart/Myocarditis	A: 2 nd ICS P: Between Trans. processes of T2-3
Bronchials/bronchitis	A: 2 rd ICS P: Trans. processes of T2
Upper Lung/pneumonia/TB/asthma	A: 3 th ICS P: Between Trans. processes of T3-4
Lower Lung/pneumonia/TB/asthma New: Pleurisy for Lower Lung Point	A: 4 th ICS P: Between Trans. Processes of T4-5

Chapman's Quick Reference

Upper/Middle GI

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Organ/Disease Examples	Anterior and Posterior Points
Esophagus/GERD/Nausea	A: 2 nd ICS P: Trans. Process T2
Stomach/Acidity/Gastric Ulcer/Neoplasm	A: 5 th ICS (L) P: Intertransverse space T5&6
Stomach/Peristalsis/Associated w/ Heart Palpitations	A: 6 th ICS (L) P: Intertransverse space T6&7
Pylorus/Pyloric Stenosis/Abdominal Bloating	A: Sternum (from junction of manubrium to junction of xyphoid) P: Trans. Process T10 (R) & 10 th Rib (R)
Pancreas/Diabetes/Glycosuria New: Pancreatic Point for Food Allergies & Bodily pH	A: 7 th ICS (R) P: Between trans. processes T7&8 (R)
Gall bladder/Gall stones/Cholecystitis New Point: Anterior/Medial Radial Head (R)	A: 6 th ICS (R) P: Between trans. processes T6&7 (R)
Liver/Enlargement	A: 6 th ICS (R) P: Between trans. processes T6&7 (R)
Liver/Torpid(Sluggish)	A: 5 th ICS (R) P: Between trans. processes T5&6 (R)
Spleen/Leukemia/Infections	A: 7 th ICS (L) P: Between trans. processes T7&8 (L)
Small Intestines/Enteritis	A: 8,9&10 th ICS P: Between trans. processes T8-9,9-10&10-11

Chapman's Quick Reference

Lower GI

Organ/Disease Examples	Anterior and Posterior Points
Appendix/Appendicitis New: 7 o'clock on right border of umbilicus (in line with McBurney's point)	A: Tip of anterior portion of 12 th rib (R) P: Trans. Process of T11 ®
Iliocecal Valve-Cecum/Colitis/Spastic Constipation	A: Greater Trochanter (R) (Upper Fifth of Femur) (Lateral Thigh) P: Trans. Process of L2,3&4 to Iliac crest (Applies to All Colon Points)
Ascending Colon/Colitis/Spastic Constipation	A: Lateral Femur (R) (Middle Femur) (Lateral Thigh) P: Same as above
Transverse Colon/Hepatic Flexure (First 2/5 th s of Transverse Colon)/Colitis/Spastic Constipation	A: Lower Femur (R) (Last Fifth of Femur) (Lateral Thigh) P: Same as above
Transverse Colon/Splenic Flexure (Last 3/5 th s of Transverse Colon) (First Lower Fifth of Femur)/Colitis/Spastic Constipation	A: Lower Femur (L) (Last Fifth of Femur) (Lateral Thigh) P: Same as above
Descending Colon /Colitis/Spastic Constipation	A: Lateral Femur (L) (Middle 3/5 th of Femur) (Lateral Thigh) P: Same as above
Sigmoid Colon/Colitis/Spastic Constipation	A: Lateral Femur (L) (Upper Femur-Greater Trochanter-L) (Upper fifth of Lateral Thigh) P: Same as above
Rectum	A: Lesser Trochanter P: Sacrum-Lower end of Iliosacral Joint)
Hemorrhoids	A: Above Ischial Tuberosities P: Sacrum-Close to ilium at Iliosacral Joint)
Atonic Constipation/Dilation	A: Between ASIS and Greater Trochanter P: Trans. Process of T11 and 11 th Rib

Chapman's Quick Reference Genitourinary

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Organ/Disease Examples	Anterior and Posterior Points
Kidneys/Bright's Ds (Edema, HTN and albuminuria)/anuria/polyuria/calculi/abscess/neoplasm	A: 1 inch lateral & 1 inch superior to umbilicus P: Between trans. process of T12&L1
Urinary Bladder/Cystitis-infection (urinary (frequency)	A: Umbilicus & Pubic Symphysis-B/T upper & lower pubes P: Upper edge of trans. process of L2
Urethra (Cystitis or calculi)	A: Inner edge of pubic ramus near upper edge of pubic symphysis P: Upper edge of trans. process of L2
New: Enuresis (Bedwetting)	Anterior Only: Pubic Symphysis

Chapman's Quick Reference Reproductive

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Organ/Disease Examples	Anterior and Posterior Points
Prostate/Prostatitis New: STD causing Prostatitis	A: Lateral Femur from Gr. Trochanter to 2 inch above knee A: Lateral margin of pubic symphysis P: Between spinous process of L5 and PSIS
Orchitis//Epididymitis/Contraction of Inguinal rings Note: This is known as the Abdominal Tension point	A: Below inguinal (Poupart's) ligament upper edge of body/ramus of pubes P: Tip of trans. process of L2
Ovaries/Ovarian cyst	A: Round ligaments from upper to lower border pubic bone P: Inner half of ovary between trans. process of T9&10 P: Outer half of ovary between trans. process of T10&11
Seminal vesicles/Vesiculitis (From STD) Fallopian Tubes//Salpingitis/Pyrosalpinx (From STD)	A: Between acetabulum and sciatic notch P: Between spinous process of L5 and PSIS
Leucorrhea/Vaginal secretions (May be from infection or normal)	A: Medial femur condyle & 3-6 inches on posterior aspect (Near knee) P: Between spinous process of L5 and PSIS
Clitoris/Priapism Vagina/Vaginismus	A: Upper/Inner/Posterior thigh 3-5 inch vertical to 1 ½-2 inches lateral P: Sacrococcygeal joint
Uterus/Inflammation	A: Upper edge junction of ramus of pubes with ischium P: Between spinous process of L5 and PSIS
Uterus/Fibroma/Neoplasm New: Breast Angle of ribs 5-6 (May be associated with uterine neoplasm)	A: Pubic symphysis to obturator foramen P: Tip of trans. process of L5 to iliac crest
Broad Ligament (of Uterus/Ovaries/Fallopian Tubes)/Abnormal Position of Uterus New: STD causing inflammation of Broad Ligament	A: Lateral femur from Gr. Trochanter to 2 inches above knee joint P: Between spinous process of L5 and PSIS
Groin Glands/Lymph nodes of femoral/inguinal area draining pelvis)/Infection	A: Inner condyle of knee lower 2/5ths of sartorius muscle to tibia P: Iliosacral joint

Chapman's Quick Reference Endocrine

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Organ/Disease Examples	Anterior and Post
Thyroid/Thyroiditis/Hyper/Hypo	A: 2 nd ICS P: Trans. process of T2
Adrenals/Fatigue	A: 1 inch lateral and 2-2 ½ inches superior to umbilicus P: Between trans. Process of T11&12
Innominate-Always treat innominate as dysfunction signify endocrinopathy Always treat innominate before any Chapman's points	A: Thickened, sensitive inguinal (Poupart's) ligament associated with a posterior innominate and or leg length discrepancy

Chapman's Quick Reference Musculoskeletal

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Organ/Disease Examples	Anterior and Posterior Points
Arms/Poor Circulation/Numbness/Tingling	A: Pectoralis minor to attachments of Ribs 3-5 P: Superior angle of scapula and ribs 1-3, especially inner margin of scapula
Upper Limb/Neuritis	A: 3 rd ICS P: Between trans. process of T3&4 and a depressed 3 rd rib (A depression of the third rib is also seen with disorders of the foot)
Upper Extremity/Hand//Dupuytren's Contracture-PARKER'S REFLEX	A: Upper, anterior, superior edge of scapula below head of humerus
Wry Neck/Torticollis/Cervical Dystonia	A: Ant. Humerus surgical neck&caudad P: Trans. Processes C3,4,6&7 New points include C2-4, T1-4 & ribs 1-2
Sciatica/Sciatic Neuritis/Lumbosacral Radiculopathy/Impingement of sciatic nerve along piriformis	A: Posterior/Outer/Lateral Femur 2-3 inches below Gr. Trochanter A: 1/5 distance above knee for 2 inches on post. Lateral femur A: 1/3 upward from condyles to mid-posterior lateral femur A: Supplemental: Superior fibula down to lateral malleolus A: Supplemental: Midway b/t trochanter and ischial tuberosities and above trochanters A: Supplemental: Below PSIS P: Upper aspect of Sacroiliac joint Note: Innominate dysfunction will be found